Montiget™

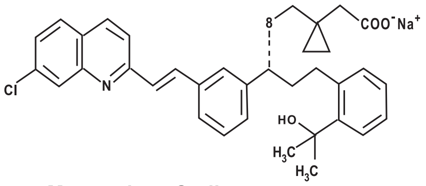
(MONTELUKAST SODIUM)

Film-coated Tablets, Chewable Tablets

DESCRIPTION

MONTIGET (Montelukast sodium) is a selective and orally active leukotriene receptor antagonist that inhibits the cysteinyl leukotriene CysLT1 receptor, stimulation of which by circulating leukotrienes is thought to play a role in the pathogenesis of asthma. It suppresses both early and late bronchoconstrictor responses to inhaled antigens or irritants, but is not suitable for the management of acute attacks of asthma.

Montelukast sodium is described chemically as [*R*-(*E*)]-1-[[[1-[3-[2-(7-chloro-2-quinolinyl)ethenyl]phenyl]-3-[2-(1-hydroxy-1-methylethyl)phenyl]propyl]thio]methyl]cyclopropaneacetic acid, monosodium salt. The molecular formula is $C_{35}H_{35}ClNNaO_3S$ and the structural formula is:



Montelukast Sodium

Montiget Chewable Tablets 4mg is light pink to pink colored, round shaped, biconvex tablet engraved “GETZ” on one side and plain on the other side.

Montiget Chewable Tablets 5mg is white colored, round shaped, biconvex tablet engraved “Getz” on one side and plain on other side.

Montiget Tablets 10mg is cream to light yellow colored square shaped film coated tablet having bisect line on one side and plain on other side.

QUALITATIVE & QUANTITATIVE COMPOSITION

MONTIGET (Montelukast sodium) is available for oral administration as:

1. MONTIGET Chewable Tablets 4mg
   Each chewable tablet contains:
   Montelukast sodium equivalent to
   Montelukast…4mg

2. MONTIGET Chewable Tablets 5mg
   Each chewable tablet contains:
   Montelukast sodium equivalent to
   Montelukast…5mg

3. MONTIGET Tablets 10mg
   Each film-coated tablet contains:
   Montelukast sodium equivalent to
   Montelukast…10mg

CLINICAL PHARMACOLOGY

Mechanism of Action

MONTIGET (Montelukast sodium) is a competitive, selective and orally active leukotriene D4 (cysteinyl leukotriene CysLT1) receptor antagonist. The cysteinyl leukotrienes (LTC4, LTD4, LTE4) are products of arachidonic acid metabolism and are released from various cells, including mast cells and eosinophils. These eicosanoids bind to cysteinyl leukotriene (CysLT) receptors. Binding of cysteinyl leukotrienes to leukotriene receptors has been correlated with the pathophysiology of asthma, including airway edema, smooth muscle contraction, and altered cellular activity associated with the inflammatory process, factors that contribute to the signs and symptoms of asthma. Thus, montelukast sodium inhibits physiologic actions of LTD4 at the CysLT1 receptors, without any agonist activity.

Pharmacokinetics

Absorption:

Montelukast sodium is rapidly absorbed following oral administration. Peak plasma concentrations of montelukast sodium are achieved in 2 to 4 hours after oral administration. The mean oral bioavailability is 64%.

Distribution:

Montelukast sodium is more than 99% bound to plasma proteins. The mean plasma half-life of montelukast sodium ranged from 2.7 to 5.5 hours in healthy young adults. The pharmacokinetics of montelukast sodium is nearly linear for oral doses up to 50mg.

Metabolism:

Montelukast sodium is extensively metabolized in the liver by cytochrome P450 isoenzymes CYP3A4, CYP2A6 and CYP2C9. Therapeutic plasma concentrations of montelukast sodium do not inhibit cytochromes P450 3A4, 2C9, 1A2, 2A6, 2C19, or 2D6.

Elimination:

The plasma clearance of montelukast sodium averages 45mL/min in healthy adults. Montelukast sodium and its metabolites are excreted principally in the feces via the bile.

Special Populations:

Elderly, Pediatric, males, females, and patients with renal insufficiency have similar plasma pharmacokinetic profiles as young adults.

Hepatic Insufficiency:

Patients with mild-to-moderate hepatic insufficiency and clinical evidence of cirrhosis has evidence of decreased metabolism and prolonged elimination half life of montelukast sodium resulting in 41% higher mean montelukast sodium area under the plasma concentration curve (AUC) following a single 10mg dose. No dosage adjustment is required in patients with mild-to-moderate hepatic insufficiency.

Pediatric Patients

In children 6 to 11 months of age, the systemic exposure to montelukast and the variability of plasma montelukast concentrations were higher than those observed in adults. Safety and tolerability of montelukast in a single-dose pharmacokinetic study in children 6 to 23 months of age were similar to that of patients two years and above.

THERAPEUTIC INDICATIONS

- MONTIGET (Montelukast sodium) is indicated for the prophylaxis and chronic treatment of asthma in adults and pediatric patients 2 years of age and older.
- Montiget is indicated in adults and pediatric patients 2 years of age and older for the relief of daytime and nighttime symptoms of seasonal allergic rhinitis.

DOSAGE AND ADMINISTRATION

The therapeutic effect of montelukast sodium on parameters of asthma control occurs within one day. MONTIGET (Montelukast sodium) tablets can be taken with or without

215mm

150mm

food. Patients should be advised to continue taking the drug while their asthma is controlled as well as during periods of worsened asthma.

MONTIGET (Montelukast sodium) should be taken once daily. For asthma, the dose should be taken in the evening. For seasonal allergic rhinitis, the time of administration may be individualized to suit patient needs.
Patients with both asthma and seasonal allergic rhinitis should take only one tablet daily in the evening.

Adults and Adolescents 15 Years of Age and Older with Asthma and/or seasonal allergic rhinitis:
The dosage for adults and adolescents 15 years old and older is one 10mg tablet daily.

Pediatric patients 6 to 14 years of age with asthma and/or seasonal allergic rhinitis:
The dosage for Pediatric patients 6 to 14 years of age is one 5mg chewable tablet daily.

Pediatric patients 2 to 5 years of age with asthma and/or sasonal allergic rhinitis:
The dosage for pediatric patients 2 to 5 years of age is one 4mg chewable tablet daily.

ADVERSE EFFECTS
Montelukast sodium is generally well tolerated. However, following are the adverse effects reported which usually were mild and did not require discontinuation of therapy.
- Hypersensitivity reactions (including anaphylaxis, angioedema, rash, pruritus, urticaria and very rarely, hepatic eosinophillic infiltration);
- Dream abnormalities, hallucinations, palpitations, drowsiness, irritability, restlessness, insomnia, increased sweating, headache;
- Nausea, vomiting, dyspepsia, diarrhea, abdominal pain;
- Myalgia including muscle cramps;
- Increased bleeding tendency, bruising edema;
- Tremor, dry mouth, vertigo, arthralgia.
- Postmarketing Experience: Blood and lymphatic system disorders: thrombocytopenia.
- Psychiatric disorders: obsessive-compulsive symptoms.

CONTRAINDICATIONS
Montelukast sodium is contraindicated in a patient who has shown hypersensitivity to the drug or any of its components. Montelukast sodium is not indicated for use in acute asthma attacks including status asthmaticus.

PRECAUTIONS
- Montelukast sodium should not be abruptly substituted for inhaled or oral corticosteroids. However, the dose of inhaled corticosteroid may be reduced gradually under medical supervision.
- Although a casual relationship with leukotriene receptor antagonism has not been established, caution and appropriate clinical monitoring is recommended when systemic corticosteroid reduction is considered in patients receiving montelukast sodium.
- Montelukast sodium should not be used as monotherapy for the treatment and management of exercise-induced asthma. Patients who have exacerbations of asthma after exercise should continue to use their usual regimen of inhaled ß-agonists as prophylaxis and should have it available as and when required.
- Montelukast sodium does not block bronchoconstrictor response to aspirin or non-steroidal anti-inflammatory drugs in aspirin-sensitive asthmatic patients. Such patients should continue to avoid aspirin and other non-steroidal anti-inflammatory drugs.
- Caution should be exercised when using montelukast sodium with bronchodilator therapy. When clinical response is apparent the bronchodilator therapy should be reduced.

Pregnancy
Montelukast sodium has not been studied in pregnant women. It should be used during pregnancy only if clearly needed.

Nursing Mothers
It is not known if montelukast sodium is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when MONTIGET is given to a nursing mother.

Drug Interactions
It is recommended that clinical monitoring, particularly in children, be conducted when potent hepatic enzyme inducers such as phenytoin, phenobarbital, or rifampicin are given with montelukast sodium. No dosage adjustment for MONTIGET (Montelukast sodium) is recommended.

Symptoms and Treatment of Overdose
There were no adverse experieces in the majority of overdose reports. The most frequently occuring adverse experiences were consistent with the safety profile of montelukast and included abdominal pain, somnolence, thirst, headache, vomiting and psychomotor hyperactivity.

STORAGE
Store below 30°C.
Protect from sunlight & moisture.
The expiration date refers to the product correctly stored at the required conditions.

HOW SUPPLIED
Montiget Chewable Tablets 4mg is available as light pink to pink colored, round shaped, biconvex tablet engraved “GETZ” on one side and plain on other side in alu-alu blister packs of 2 x 7’s and 4 x 7’s.

Montiget Chewable Tablets 5mg is available as white colored, round shaped, biconvex tablet engraved “Getz” on one side and plain on other side in alu-alu blister packs of 2 x 7’s and 4 x 7’s.

Montiget Tablets 10mg is available as cream to light yellow colored square shaped film coated tablet having bisect line on one side and plain on other side in alu-alu blister packs of 2 x 7’s and 4 x 7’s.

Keep out of reach of children

Date of revision: September 2019

Please read the contents carefully before use.
This package insert is continually updated from time to time.

Getz pharma (PVT) LIMITED
A member of The Getz Group, USA.

MLY-200000000

Manufactured by: Getz Pharma (Pvt.) Limited, 29-30/27, K.I.A., Karachi - 74900, Pakistan.

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